

UnitedHealthcare Pharmacy
Clinical Pharmacy Programs

Program Number	2026 P 2168-11
Program	Prior Authorization/Medical Necessity
Medication	Spravato® (esketamine)
P&T Approval Date	6/2019, 9/2019, 2/2020, 12/2020, 4/2021, 4/2022, 12/2022, 12/2023, 6/2024, 3/2025, 4/2026
Effective Date	7/1/2026

1. Background:

Spravato® (esketamine) is a non-competitive N-methyl D-aspartate (NMDA) receptor antagonist indicated for the treatment of:

- treatment-resistant depression (TRD) in adults, as monotherapy or in conjunction with an oral antidepressant.
- depressive symptoms in adults with major depressive disorder (MDD) with acute suicidal ideation or behavior in conjunction with an oral antidepressant.

Limitations of Use: The effectiveness of Spravato in preventing suicide or in reducing suicidal ideation or behavior has not been demonstrated. Use of Spravato does not preclude the need for hospitalization if clinically warranted, even if patients experience improvement after an initial dose of Spravato. Spravato is not approved as an anesthetic agent. The safety and effectiveness of Spravato as an anesthetic agent have not been established.

Because of the risks of serious adverse outcomes resulting from sedation, dissociation, and abuse and misuse, Spravato is only available through a restricted program under a Risk Evaluation and Mitigation Strategy (REMS) called the Spravato REMS.

For the purposes of this program, a trial and failure of a given antidepressant is defined as the patient unable to achieve a clinical meaningful improvement of the maximally tolerated dose(s) for at least 8 weeks.

2. Coverage Criteria ^a:

A. Major depressive disorder (treatment-resistant)

1. Initial Authorization

- a. **Spravato** will be approved based on all of the following criteria

- (1) Diagnosis of major depressive disorder (treatment-resistant), according to the current DSM (i.e., DSM-5-TR) criteria, by a mental health professional

-AND-

- (2) Submission of medical records (e.g., chart notes, laboratory values) documenting baseline scoring (prior to starting Spravato) on at least one of the following clinical assessments has been completed:

- (a) Beck Depression Inventory (BDI)
- (b) Hamilton Rating Scale for Depression (HAMD)
- (c) Montgomery-Asberg Depression Rating Scale (MADRS)
- (d) 9-item Patient Health Questionnaire (PHQ-9)
- (e) Quick Inventory of Depressive Symptomatology (QIDS)

-AND-

- (3) History of failure of a trial^b of at least **two** different antidepressant medications or treatment regimens for a duration of at least 8 weeks each (document medication, date, and duration of trial)

An antidepressant or treatment regimen would include any of the following classes or combinations:

- (a) Selective serotonin reuptake inhibitors (e.g., citalopram, fluoxetine, paroxetine, sertraline)
- (b) Serotonin norepinephrine reuptake inhibitors (e.g., duloxetine, venlafaxine, etc.)
- (c) Bupropion
- (d) Tricyclic antidepressants (e.g., amitriptyline, clomipramine, nortriptyline, etc.)
- (e) Mirtazapine
- (f) Monoamine oxidase inhibitors (e.g., selegiline, tranylcypromine, etc.)
- (g) Serotonin modulators (e.g., nefazodone, trazodone, etc.)
- (h) Augmentation with antipsychotics, lithium, or thyroid hormone

-AND-

- (4) Provider and/or the provider's healthcare setting is certified in the Spravato REMS program

-AND-

- (5) Prescribed by or in consultation with a psychiatrist

Authorization will be issued for 12 months.

2. Reauthorization

- a. **Spravato** will be approved based on **all** of the following criteria:

- (1) Documentation of remission or a positive clinical response to Spravato therapy

-AND-

- (2) Submission of medical records (e.g., chart notes, laboratory values) documenting baseline and recent (within the last month) scoring on at least **one** of the following assessments demonstrating remission or clinical response (e.g., score reduction from baseline):

- (a) BDI
- (b) HAMD
- (c) MADRS
- (d) PHQ-9
- (e) QIDS

-AND-

- (3) Provider and/or the provider's healthcare setting is certified in the Spravato REMS program

-AND-

- (4) Prescribed by or in consultation with a psychiatrist

Authorization will be issued for 12 months.

B. Depressive symptoms in adults with major depressive disorder (MDD) with acute suicidal ideation or behavior

1. Authorization

- a. **Spravato** will be approved based on all of the following criteria:

- (1) Diagnosis of major depressive disorder according to the current DSM (i.e., DSM-5-TR) criteria, by a mental health professional

-AND-

- (2) Patient is experiencing an acute suicidal ideation or behavior

-AND-

- (3) Spravato will be used in combination with a newly initiated or optimized oral antidepressant

-AND-

- (4) Provider and/or the provider's healthcare setting is certified in the Spravato REMS program

Authorization will be issued for 12 months.

^a State mandates may apply. Any federal regulatory requirements and the member specific benefit plan coverage may also impact coverage criteria. Other policies and utilization management programs may apply.

^b For Connecticut, Kentucky and Mississippi business, only a 30-day trial will be required.

3. Additional Clinical Rules:

- Notwithstanding Coverage Criteria, UnitedHealthcare may approve initial and re-authorization based solely on previous claim/medication history, diagnosis codes (ICD-10) and/or claim logic. Use of automated approval and re-approval processes varies by program and/or therapeutic class.
- Supply limits may be in place.

4. References:

1. Spravato [prescribing information]. Lakewood, NJ; Janssen Pharmaceuticals, Inc.; April 2025.
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3. American Psychiatric Association (APA). Practice guideline for the treatment of patients with major depressive disorder. 3rd ed. Arlington (VA): American Psychiatric Association (APA); 2010 Oct.
4. Psychopharmacologic Drugs Advisory Committee and Drug Safety and Risk Management Advisory Committee Meeting. FDA Briefing Document. February 12, 2019. <https://www.fda.gov/downloads/AdvisoryCommittees/CommitteesMeetingMaterials/Drugs/PsychopharmacologicDrugsAdvisoryCommittee/UCM630970.pdf>. (Accessed on March 11, 2019)
5. Thase M, Connolly KR. Unipolar depression in adults: Choosing treatment for resistant depression. Solomon D, ed. UpToDate. Waltham, MA: UpToDate Inc. <https://www.uptodate.com> (Accessed on February 16, 2026)
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9. Trivedi MH, Rush AJ, Wisniewski SR, et al. Evaluation of outcomes with citalopram for depression using measurement-based care in STAR*D: implications for clinical practice. *Am J Psychiatry*. 2006 Jan;163(1):28-40.
10. Trivedi MH, Rush AJ, Gaynes BN, et al. Maximizing the adequacy of medication treatment in controlled trials and clinical practice: STAR*D measurement-based care. *Neuropsychopharmacology*. 2007 Dec;32(12):2479-89.
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12. Popova V, Daly EJ, Trivedi M, et al. Efficacy and Safety of Flexibly Dosed Esketamine Nasal Spray Combined With a Newly Initiated Oral Antidepressant in Treatment-Resistant Depression: A Randomized Double-Blind Active-Controlled Study. *Am J Psychiatry*. 2019 Jun 1;176(6):428-438.
13. Fu DJ, Ionescu DF, Li X, et al. Esketamine Nasal Spray for Rapid Reduction of Major Depressive Disorder Symptoms in Patients Who Have Active Suicidal Ideation With Intent: Double-Blind, Randomized Study (ASPIRE I). *J Clin Psychiatry*. 2020 May 12;81(3):19m13191.
14. Ionescu DF, Fu DJ, Qiu X, et al. Esketamine Nasal Spray for Rapid Reduction of Depressive Symptoms in Patients With Major Depressive Disorder Who Have Active Suicide Ideation With Intent: Results of a Phase 3, Double-Blind, Randomized Study (ASPIRE II), *International Journal of Neuropsychopharmacology*, pyaa068

15. Coley RY, Boggs JM, Beck A, Hartzler AL, Simon GE. Defining Success in Measurement-Based Care for Depression: A Comparison of Common Metrics. *Psychiatr Serv.* 2020;71(4):312-318. doi:10.1176/appi.ps.201900295
16. Taylor RW, Marwood L, Oprea E, et al. Pharmacological Augmentation in Unipolar Depression: A Guide to the Guidelines. *Int J Neuropsychopharmacol.* 2020;23(9):587-625. doi:10.1093/ijnp/pyaa033

Program	Prior Authorization/Medical Necessity – Spravato (esketamine)
Change Control	
6/2019	New program.
9/2019	Updated coverage criteria.
2/2020	Updated coverage criteria to require submission of baseline validated provider administered assessments.
12/2020	Updated background and coverage criteria for new indication for MDD with acute suicidal ideation or behavior. Updated references.
4/2021	Clarified submission of clinical assessments to include medical record submission.
4/2022	Annual review with no change to clinical criteria.
12/2022	Updated coverage criteria for treatment-resistant depression with clarification of requirement for combination with “new” oral antidepressant to be an agent that has not been previously failed.
12/2023	Annual review. Added PHQ-9 scale to list of options for clinical assessments. Updated background and references.
6/2024	Updated wording of coverage criteria without change to clinical intent. Updated approval duration, both initial and reauthorization, to 12 months.
3/2025	Revised options for clinical assessments to reflect different item versions of the same scale as well as added BDI. Removed requirement for combination with oral antidepressant for TRD per updated label. Revised coverage criteria for TRD to require history of failure of a trial of at least two different antidepressant medications or treatment regimens, remove reference to current depressive episode, and remove augmentation with anticonvulsants as a treatment regimen based on latest clinical evidence. Updated references.
4/2026	Annual review. Updated references.